

NEW

2026

Ethiopian Health Profession Licensing Examination(EHPLE)

INFORMATION BOOKLET

PHYSIOTHERAPY



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MINISTRY OF HEALTH - ETHIOPIA

የዜጎች ጤና ለሃገር ብልጽግና!
HEALTHIER CITIZENS FOR PROSPEROUS NATION!



AAU-Testing Center

Message From the State Minister, Ministry of Health -Ethiopia



H.E Mrs. Frehiwot Abebe

Improving healthcare quality is a global priority for sustainable development, with high quality healthcare being a key component of universal health coverage. One strategy to maintain health care standards is through provision of health professionals' competency assessment. Consequently, in 2019, the Ministry of Health-Ethiopia, initiated the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in seven health disciplines, which has since expanded to include 17 health disciplines.

The main goal of this competency assessment is to identify health professionals with minimal competencies necessary to perform their duties safely and competently, thus enhancing the quality of health care services. This initiative is overseen by a dedicated Health and Health Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), comprising four desks, which plays a pivotal role in strengthening the system and enabling the LEO to conduct the competency exam more extensively and with improved organization and quality.

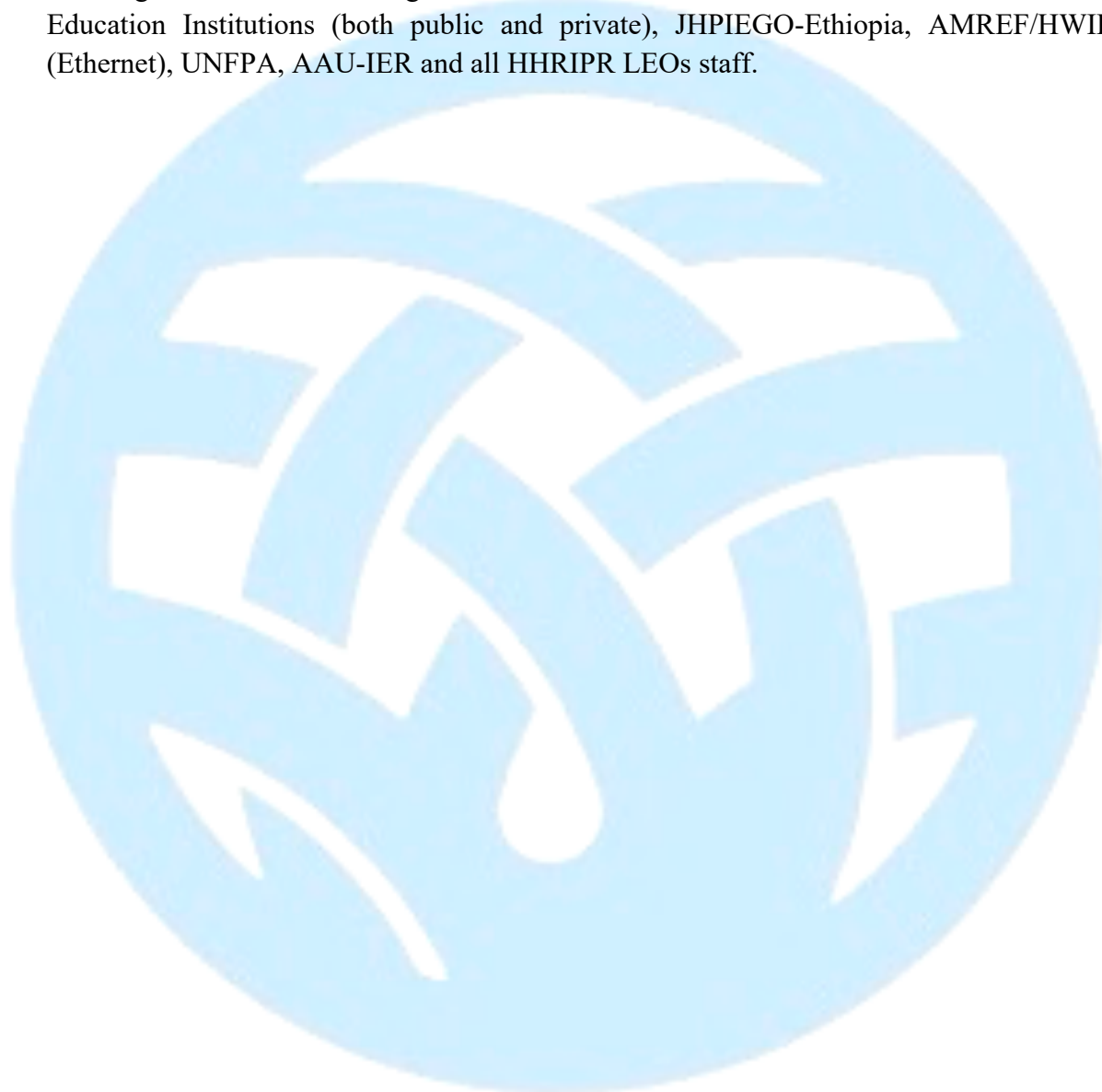
It is important to note that this competency assessment differs significantly from traditional academic or employment examinations. Hence, this information booklet has been created to address the informational needs of both examinees and teaching faculty regarding the EHPLE. Additionally, it aims to facilitate the assessment process, while promoting transparency and ensuring the sustainability of the program.

The preparation of this booklet involved the collaboration of esteemed experts from various higher education institutions, AAU-IER, the Ministry of Health, JHPIEGO-Ethiopia, Amref, Health Professionals 'Associations, and the Ministry of Education. Their invaluable contributions are acknowledged with sincere gratitude, alongside appreciation for the Ministry of Health staff for their unwavering commitment and hard work throughout the project.

Acknowledgements

This Information Booklet for EHPLE is a contribution from several educators, researchers, students and concerned individuals with a genuine interest to propel Ethiopia's medical and health sciences education forward.

The Ministry of Health is grateful for the contribution of many individuals and institutions in realizing this endeavor. Among these are Professional Associations, Student Association, Higher Education Institutions (both public and private), JHPIEGO-Ethiopia, AMREF/HWIP, MoE (Ethernet), UNFPA, AAU-IER and all HHRIPR LEOs staff.



Acronyms and Abbreviations

EHPLE	Ethiopian Health Profession Licensing Examination
ETA	Educational and Training Authority
HEIs	Higher Education Institutions
HHrIPR-LEO	Health and Health-related Institutions and Professionals Regulatory Lead Executive Office
HSTP-II	Health Sector Transformational Plan-II
MCQ	Multiple Choice Question
MOH	Ministry of Health
WHO	World Health Organization

Purpose of the Information Booklet

The EHPLE Information Booklet serves as a comprehensive guide for those individuals seeking information about the exam. It typically outlines basic information for candidate registration, exam development and administration processes and procedures, result notification, and the licensing process. It also includes information on the exam framework, i.e., the exam domain, sub-domain, content, process, and task, with sample exam items specific to each profession.

The publication of this Booklet is crucial for the following reasons:

- **Clarity and guidance:** It provides clear information about the exam by ensuring candidates understand the necessary information to prepare them.
- **Accessibility:** It serves as a readily accessible resource for individuals pursuing to take the exam, consolidating essential information in one document and facilitating easy access to necessary details. It also helps other stakeholders who might be interested in such resources.
- **Transparency:** It promotes transparency in the examination process and fosters trust among stakeholders about the exam.

In summary, the publication of this Booklet is essential for creating a transparent, standardized, and accessible framework that guides candidates through the EHPLE process.

Contents

Message From the State Minister, Ministry of Health -Ethiopia	2
Acknowledgements	3
Acronyms and Abbreviations	4
Purpose of the Information Booklet	5
Definition of terms	7
1. Introduction	1
1.1. Background	1
1.2. The Rationale of EHPLE	1
2. Key processes of EHPLE	2
2.1. Registration of candidates	2
2.2. Task Analysis	3
2.3. Exam Blueprint	3
2.4. Item Development	3
2.5. Item Review	3
2.6. Standard setting method	4
2.7. Exam Administration	4
2.8. Scoring and post exam analysis	5
2.9. Result notification and appeal management	5
2.10. Licensing	5
3: Exam Framework	6
Sample questions	13
List of contributors	21
Contact Us	21

Definition of terms

- **Domain:** a broad category or area of knowledge or skills of a profession
- **Sub-domain:** a subset of a broader domain that focuses on knowledge or skills related to the overarching domain
- **Content:** a more specific subcategory, which is a breakdown of the sub-domain
- **Task:** the responsibility, knowledge, skill, and attitude of a junior undergraduate professional in an actual work environment
- **Process:** a systematic sequence of steps or actions designed to achieve a specific outcome
- **Learning outcome:** a clear and measurable statement that describes what the examinee is expected to know or be able to do
- **Relative emphasis:** the proportional importance or weight assigned to different content areas or categories within the assessment
- **Item:** a particular multiple-choice question
- **Item developer:** a subject matter expert responsible for writing test items or questions that make up the examination
- **Item reviewer:** a subject matter expert responsible for reviewing and refining the test items or questions that make up the examination
- **Standard setting:** a process of determining a cut-off point or passing score for an exam
- **Item difficulty index:** a statistical measure that indicates the proportion of examinees who answered a particular test item correctly
- **Discrimination index:** a statistical measure that evaluates how well a particular test item differentiates between high-performing and low-performing examinees
- **Admission paper:** a printout paper generated by the system after completing registration that contains the examinee's photo, QR code, and necessary information

1. Introduction

1.1. Background

Competency assessment is one of the strategies for controlling the standard of healthcare services provided in healthcare facilities. The World Health Organization (WHO) recommends all healthcare professionals to have necessary competencies. In Ethiopia, the Health Sector Transformational Plan-II (HSTP-II) states competency assessment of all graduates before joining the health workforce as one of the strategic initiatives.

The Ministry of Health (MOH) launched the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in 2019. The Health and Health-related Institutions and Professionals Regulatory Lead Executive Office (HHrIPR-LEO) of the Ministry of Health is given a mission to implement the Ministry's objective to achieve competency-related goals. It has the responsibility to ensure that the EHPLE meets technical, professional, and legal standards, and to protect the health, safety, and welfare of the public by assessing candidates' abilities to practice competently.

Currently, the exam is given for 17 health professions (Medicine, Nursing, Public Health, Pharmacy, Medical Laboratory Science, Anesthesia, Midwifery, Dental Medicine, Medical Radiology Technology, Environmental Health, Psychiatric Nursing, Pediatric and Child Health Nursing, Emergency and Critical Care Nursing, Human Nutrition, Surgical Nursing, Optometry, and Physiotherapy). Since its introduction until December 2025, a total of 204,811 examinees took the exam in 23 rounds.

1.2. The Rationale of EHPLE

One of the critical functions of the MoH is to guarantee the efficiency, quality, and equity of healthcare delivery and to protect the public from any undesirable consequences in healthcare delivery practices. As professionals' competence is a significant determinant of the quality of health, evaluation of health professionals' competence has now been given due attention. The licensing examination for health professionals serves as a crucial step to ensure that individuals entering the field meet specific competency standards. The sole aim of the competency assessment is to safeguard public health by verifying that health professionals have the minimal basic knowledge, attitude, and skill required to provide safe and effective care.

Licensing exams act as a preventive measure, ensuring that only competent professionals join the health workforce, which, in turn, contributes to reducing the occurrence of medical errors and enhancing overall patient safety. By setting standards through examinations, regulatory bodies strive to minimize the risk of medical errors caused by incompetence.

2. Key processes of EHPLE

EHPLE involves several key processes to ensure the quality and reliability of the examination.

2.1. Registration of candidates

EHPLE has a mandatory online registration system for both new and repeat candidates, which can be found at <http://hple.moh.gov.et>

Please note these important notes during registration.

New Test Takers:



- The list of eligible candidates from governmental and private Higher Education Institutions (HEIs) will be uploaded to MoH's online registration system by the registrar office of their respective HEIs.
- Candidates who completed their education abroad must bring the following documents in person to MoH in order for their names to be uploaded to the online registration system:
 - ✓ Equivalence document from ETA
 - ✓ Externship attachment completion letter after completing the externship assigned by the regulatory body
- Once the name of the candidate is uploaded to the system and registration has opened for the current exam round, the candidate must register at <http://hple.moh.gov.et> by uploading the necessary documents listed below.
 - ✓ a scanned original or temporary degree
 - ✓ a scanned national ID

Repeat Test Takers:



- Since the information about re-exam candidates already exists in the system, the candidate should register by directly going to <http://hple.moh.gov.et>. There is no need for re-exam candidates to upload their documents.

Both new and repeat candidates:



- After completing the registration, the candidate must download and print the admission paper by logging into his/her account using username and password, or national ID FAN.
- The candidate can change the exam center by logging into his/her account only during the registration period
- An application for center change is not allowed after the registration period is closed.

2.2. Task Analysis

The first step of exam development involves conducting a comprehensive task analysis study, which identifies the tasks, knowledge, skills, and abilities required from a junior undergraduate professional in the specific profession. The analysis is typically done through surveys, interviews, or observations of practitioners in the actual work environment, as well as through the Delphi method with subject matter experts.

2.3. Exam Blueprint

Based on the task analysis findings, the exam blueprint is prepared that outlines the content areas to be covered in the examination and the weight or emphasis given to each area. This ensures that the exam reflects the key competencies and knowledge needed for competent practice in that specific profession. Blueprint is the matrix or chart that shows the number and type of test questions represented across the topics in the content area, consistent with the learning outcome and relative weight of the test given to each content area. The blueprint also identifies the percentage weighting of cognitive dimensions as the level of competence tested in each knowledge domain.

Key components of blueprint are:

- Domain
- Sub-domain
- Content
- Task
- Process
- Learning outcome
- Assessment methods
- Assessment tools/instrument (test format)
- Relative emphasis (in percentage)

2.4. Item Development

The items are developed following specific guidelines to ensure clarity, relevance, and fairness. Subject matter experts with experience in the field are selected from HEIs to develop exam questions (items) that align with the exam blueprint. The exam questions focus mainly on “knows how” according to the competency level of the Miller's pyramid. The items are produced in a secure location on designated computers that are free from internet connectivity. The items are scenario-based and constructed with stem, lead-in, and four options/alternatives. All items will have a single-best-answer type of Multiple-Choice Question (MCQ) that addresses the learning outcome defined in each content area. Standard textbooks, updated guidelines, and standards are used as reference materials.

2.5. Item Review

Once developed, the items undergo a rigorous review process by item reviewers. The main purpose of the exam review process is to evaluate content relevance, technical accuracy, clarity, and sensitivity related to culture and religion. More experienced subject matter experts as well as psychometric experts will do the review to ensure the items meet psychometric standards. Subject matter experts shall review the items to confirm that they are accurate, clearly stated, and correctly keyed using the checklist. Psychometric experts shall review the items to ensure that

they are not technically flawed. They also work on editorial review to check grammar, punctuation, and spelling errors. This helps ensure the reliability and validity of the items.

2.6. Standard setting method

The standard setting or cut-off point of the EHPLE is determined using the Modified-Angoff method, which is one of the most widely used and legally defensible standard setting approaches to set a cut-off point for high-stake competency examinations.

The method involves a panel of subject matter experts who evaluate each exam question and then estimate the probability that a minimally competent examinee would answer each test item correctly. The average of the experts' predictions for an exam question becomes its predicted difficulty. The average of the predicted difficulty values across all items on a test is the recommended cut-off point. This point indicates the minimum level of knowledge and skill required to pass.

2.7. Exam Administration

The EHPLE is administered following established protocols and guidelines. Proper exam administration procedures, appropriate security measures, and appropriate consideration for test-takers who need special support will be applied during exam administration at exam centers. The exam is administered in selected HEIs nationally, where candidates can choose based on their convenience at the time of registration. The exam schedule will be posted ahead of time on the MoH website and social media pages. Examinees who have a valid admission paper are eligible to sit for the exam. The mode of exam administration is computer-based testing.

CAUTIONS

-
- Candidates are allowed



- Attend the orientation session in order to sit for the exam
 - Arrive at the exam center on time
 - Bring a legal ID and admission paper
 - Complete the exam within the allotted time frame
-

➤ Candidates are **NOT** allowed



- To bring reference materials, blank paper, or notes into the exam center
- To smoke, eat, or drink in the exam room
- To bring mobile phones, tablets, smart watches, camera devices, eyeglasses, calculators or any type of electronic device into the exam center
- To bring their personal belongings to the exam center
- To bring weapons and sharp materials into the exam center
- To give or receive assistance to or from other candidates during the examination

2.8. Scoring and post exam analysis

Once the exam is completed, the scoring process begins. The exam scoring process involves computerized scoring using software.

Post-exam analysis is the process of analyzing examinees' responses to individual test items in order to assess the quality of the items and the exam as a whole. This phase helps to identify any poorly performing items that may need revision or removal from the exam. The item difficulty index, discrimination index, and reliability coefficient are elements of exam analysis.

2.9. Result notification and appeal management

After scoring and analysis, individual score reports are generated and provided to examinees through the website <http://hple.moh.gov.et>. After result notification, examinees can submit their appeal through phone or email within 10 working days after result notification.

2.10. Licensing

The list of examinees who passed the exam will be sent to regional and city administration regulatory bodies. A license is obtained from the regional/zonal health bodies where he/she permanently lives.

Requirements for professional licensing are:



- Passing the EHPLE
- Original or temporary degree
- Educational documents (10th and 12th certificates)
- Medical certificate
- Government issued ID
- Additional prerequisites based on the requirements of regional regulatory bodies

3: Exam Framework

The key broader professional roles, also known as domains or main knowledge areas serve as a building framework for the licensing examination content for *Physiotherapy professionals*. The domains are further divided into discrete professional attributes that constitute sub-units (also referred to as sub-domains) defining the professional identity of for *Physiotherapy professionals*. specifying the performance level of each sub-domain serve as the final characteristic of the professional duties on which the licensing exam focuses.

The contents of the licensing examination are presented below, structured into key roles (domains), sub-units (sub-domains), and tasks. The examination emphasis for each domain and sub-domain, out of the total 100% questions, is indicated in brackets.

Key professional roles/ domains

- ☐ Patient Care (82.5%)
- ☐ Scholar (5.0%)
- ☐ Professionalism (5.0%)
- ☐ Leadership and management (4.0%)
- ☐ Health promotion and disease prevention (3.5%)

Key role/ domain 1: Patient care (82.5%)

Description: This domain encompasses the professional roles of Physiotherapists in the provision of high-quality, safe, and patient-centered holistic physiotherapy care within their scope of practice. The provision of up-to-date, ethical, and resource-efficient physiotherapy care requires the application of integrated knowledge of biomedical, fundamental physiotherapy, clinical, behavioral, and social sciences. As patient care providers, physiotherapists should implement the physiotherapy care process by collecting and interpreting information, making clinical decisions, carrying out diagnostic and therapeutic physiotherapy interventions, providing physiotherapy care, and evaluating interventions. To demonstrate competence in this domain, candidates shall apply such integrated knowledge in the following sub-areas:

- ☐ MSK physiotherapy condition (22.5%)
- ☐ Pediatric physiotherapy condition (15%)
- ☐ Neurology physiotherapy condition (18.5%)
- ☐ Trauma, Orthopedics and surgery (10.5%)
- ☐ Women's Health (3%)
- ☐ Geriatrics (2%)
- ☐ Cardiopulmonary Physiotherapy (4.5%)
- ☐ Sport related injuries (2.5%)
- ☐ Prostheses, Orthosis and Mobility aid technologies (3%)
- ☐ Palliative care (1%)

Key role/ domain 2: Scholar (5.0%)

Description: This domain encompasses the professional roles of *physiotherapy professionals* in generating and utilizing scientific data to improve the health and well-being of the community and broaden their scientific knowledge within the *healthcare system and community setting*. Providing this service requires the application of integrated knowledge in research methods, measurements of health and disease, biostatistics, epidemiology, clinical audit, evidence-based practice, and research ethics. To demonstrate competence in this domain, candidates must possess applied knowledge in planning and problem identification, data collection, analysis, interpretation, report write-up, and dissemination of research outputs.

Key role/ domain 3: Professionalism (5.0%)

Description: This domain encompasses the professional commitment of *Physiotherapy professionals* to promoting the health and well-being of individuals and society through adhering to ethical standards, maintaining personal integrity, and upholding high standards of competence in all areas of *practice*. To exhibit competence in this domain, candidates must possess applied knowledge of ethical principles, medico-legal practices, effective communication, accountability to the profession and society, maintenance of professional excellence and personal health, and professional values such as compassion, respect, integrity, honesty, altruism, and humility.

Key role/ domain 4: Leadership and management (4.0%)

Description: This domain encompasses the professional roles of *Physiotherapy professionals* in envisioning a high-quality healthcare system through self-awareness, active participation in healthcare teams, leading teams, and managing health systems. Providing this service requires the application of integrated knowledge in continuous quality improvement, effective health system leadership, management, and healthcare ethics. To demonstrate competence in this domain, candidate shall possess applied knowledge to plan, organize, staff, lead, execute, monitor, and control healthcare resources and activities.

Key role/ domain 5: Health promotion and disease prevention (3.5%)

Description: This domain encompasses the professional roles of *Physiotherapy professionals* in enhancing the health and well-being of patients, communities, and the larger populations they serve through health advocacy, disease prevention, health promotion and the promotion of health equity. Providing this service takes an integrated understanding of determinants of health, health informatics, epidemiology, communicable disease control, health education.

Table 1 Exam content for Physiotherapy profession

Domain 1: Patient Care	
Sub-domain 1.1 MSK	
Content	Task
MSK related to Spine, pelvis and head conditions	Assess patients with MSK related Spine, head and Pelvic conditions
	Formulate an appropriate diagnosis for MSK related Spine, head and Pelvic conditions
	Manage patients with MSK related Spine, head and Pelvic conditions (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercises)
MSK related Upper limb conditions	Assess patients with MSK related Upper limb conditions
	Diagnose patients with MSK related Upper limb conditions
	Manage patients with MSK related Upper limb conditions (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercises)
MSK related to lower limb conditions	Assess patients with MSK related lower limb conditions
	Formulate an appropriate diagnosis for MSK related lower limb conditions
	Manage patients with MSK related lower limb conditions (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Sub-domain 1.2: Pediatrics	
Content	Task
Developmental & neurological disorders	Assess pediatric developmental & neurological disorders
	Diagnose pediatric Developmental & neurological disorders
	Manage pediatric developmental & neurological disorders (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Congenital disorders (MSK related and cardiovascular conditions)	Assess Pediatric Congenital disorders
	Formulate an appropriate diagnosis for Pediatric Congenital disorders
	Manage Pediatric Congenital disorders (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Disorders of bone, joints muscles and skin	Assess Pediatric disorders of bone, joints muscles and skin
	Formulate an appropriate diagnosis for Pediatric disorders of bone, joints muscles and skin

	Manage Pediatric disorders of bone, joints muscles and skin (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Sub-domain 3: Neurology	
Content	Task
Infectious disease of NS	Assess patients with Infectious disease of Nervous System (NS)
	Formulate an appropriate diagnosis for patients with Infectious disease of Nervous System
	Manage Patients with Infectious disease of Nervous System (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Traumatic Injuries of NS (TBI, SCI, ...)	Assess patients with traumatic Injuries of NS
	Formulate an appropriate diagnosis for patients with traumatic Injuries of NS
	Manage patients with traumatic Injuries of NS (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
CNS lesions (tumor, CVA....)	Assess patients with Central nervous system lesions
	Formulate an appropriate diagnosis for patients with Central nervous system lesions
	Manage patients with Central nervous system lesions (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Neurodegenerative disorders (Parkinson disease,)	Assess patients with Neurodegenerative disorders
	Formulate an appropriate diagnosis for patients with Neurodegenerative disorders
	Manage patients with Neurodegenerative disorders (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Peripheral Nervous System (PNS) disorders (like motor neuron diseases, leprosy...)	Assess patients with Peripheral Nervous System disorders
	Formulate an appropriate diagnosis for patients with Peripheral Nervous System disorders
	Manage patients with Peripheral Nervous System disorders (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Sub-domain 1.4: Trauma, Orthopedics and surgery	
Content	Task
Traumatic conditions (Fractures, Dislocations /subluxations, amputation and burns)	Assess patients with Traumatic conditions (Fractures, Dislocations /subluxations, amputation and burns)
	Diagnose Patients with Traumatic conditions (Fractures, Dislocations /subluxations, amputation and burns)

	Manage Patients with Traumatic conditions such as Fractures, Dislocations /subluxations, amputation and burns. (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
chronic and Inflammatory conditions of the joints (OA, OP, rickets, OM, RHA,)	Assess patients with chronic and Inflammatory conditions of the joints (OA, OP, rickets, OM, RHA)
	Formulate an appropriate diagnosis for patients with chronic and Inflammatory conditions of the joints (OA, OP, rickets, OM, RHA,)
	Manage patients with chronic and Inflammatory conditions of the joints like OA, OP, rickets, OM, RHA. (therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Pre & Post surgical conditions	Assess patients with Pre & Post surgical conditions (abdominal, joint ...)
	Formulate an appropriate diagnosis for patients with Pre & Post surgical conditions (abdominal, joint ...)
	Manage patients with Pre & Post surgical conditions using therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise
Sub-domain 1.5: Women's Health	
Content	Task
Pregnancy and birth related Neuromusculoskeletal conditions (carpal tunnel syndrome, low back pain, footdrop, pelvic girdle pain, diastasis recti abdomeni.)	Assess patients with pregnancy and birth related Neuromusculoskeletal conditions (carpal tunnel syndrome, low back pain, footdrop, pelvic girdle pain, diastasis recti abdomeni.)
	Formulate an appropriate diagnosis for patients with pregnancy and birth related Neuromusculoskeletal conditions (carpal tunnel syndrome, low back pain, footdrop, pelvic girdle pain, diastasis recti abdomeni.)
	Manage patients with pregnancy and birth related Neuromusculoskeletal conditions (carpal tunnel syndrome, low back pain, footdrop, pelvic girdle pain, diastasis recti abdomeni.) using therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise
Urogynecology conditions and post-surgical conditions (pelvic organ prolapse, incontinence, pelvic floor dysfunction,)	Assess patients with Urogynecology conditions and post-surgical conditions (pelvic organ prolapse, incontinence, pelvic floor dysfunction,)
	Formulate an appropriate diagnosis for patients with Urogynecology conditions and post-surgical conditions (Pelvic organ prolapse, incontinence, pelvic floor dysfunction,)
	Manage patients with Urogynecology conditions and post-surgical conditions (pelvic organ prolapse, incontinence, pelvic floor dysfunction,...) using therapeutic exercise , manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise
Sub-domain 1.6: Geriatrics	
Content	Task

Age related complications (physical, mobility limitations, cognitive, psychiatric,)	Assess patients with Age related complications (physical, mobility limitations, cognitive, psychiatric)
	Formulate an appropriate diagnosis for patients with Age related complications (physical, mobility limitations, cognitive, psychiatric,)
	Manage patients with Age related complications (physical, mobility limitations, cognitive, psychiatric,) using therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise
Subdomain 1.7: Cardiopulmonary Physiotherapy	
Content	Task
Chronic Cardiac conditions (Coronary artery disease, Angina, Congenital heart disease, PAD, DVT, Congestive HF, myocardial infarction)	Assess patients with Chronic Cardiac conditions (Coronary artery disease, Angina, Congenital heart disease, Peripheral arterial diseases, deep vein thrombosis, Congestive heart failure, myocardial infarction)
	Formulate an appropriate diagnosis for Chronic Cardiac conditions (Coronary artery disease, Angina, Congenital heart disease, Peripheral arterial diseases, deep vein thrombosis, Congestive heart failure, myocardial infarction)
	Manage Chronic Cardiac conditions (Coronary artery disease, Angina, Congenital heart disease, Peripheral arterial diseases, deep vein thrombosis, Congestive heart failure, myocardial infarction) using cardiac rehabilitation program
Pulmonary conditions (TB, Pneumonia, Restrictive PD, Chronic Obstructive Pulmonary Disease)	Assess patients with Pulmonary conditions (TB, Pneumonia, Restrictive PD, Chronic Obstructive Pulmonary Disease)
	Formulate an appropriate diagnosis for Pulmonary conditions (TB, Pneumonia, Restrictive PD, Chronic Obstructive Pulmonary Disease)
	Manage patients with Pulmonary conditions (TB, Pneumonia, Restrictive PD, Chronic Obstructive Pulmonary Disease)
Patients in ICU	Conduct proper assessment patients in ICU
	Formulate an appropriate diagnosis for Patients in ICU
	Manage Patients in ICU
Sub-domain 1.8: Sport related injuries	
Content	Task
Sport related injuries	Conduct proper assessment for patients with Sport related injuries
	Formulate an appropriate diagnosis for Sport related injuries
	Manage patients with sport related injuries using therapeutic exercise, manual therapeutic techniques, Electrotherapy modalities and exercise therapy equipment and functional exercise)
Sub-domain 1.9: Prosthesis, Orthosis and Mobility aid technologies	
Content	Task
Prosthesis, Orthosis and Mobility aid technologies	Assess Prosthesis, Orthosis and Mobility aids users
	Manage patients with prosthesis, Orthosis and Mobility aids users
Sub-domain 1.10: Palliative care	

Content	Task
Palliative care	Assess patients in the Palliative care
	Provide Physiotherapy care in palliative care setting

Domain 2: Leadership and management

Tasks

Apply basic principles and concepts of management

Domain 3: Scholar

Tasks

Designing research proposal

Analyzing data

Writing project work

Applying evidence-based practice

Domain 4: Professionalism

Tasks

Apply ethical principles and legal standards in professional practice and community service

Participating in collaborative work

Apply effective communication strategies

Domain 5: Health promotion and disease prevention

Tasks

Provide health education on major community health problems of physiotherapy concern (DM, HTN, obesity, physical inactivity)

Participate in public health emergency preparedness and response

Provide health education on major ergonomic problems

Provide community-based rehabilitation services

Sample questions

1. A 45-year-old female patient came to a hospital with a complaint of severe lower back pain that radiates to the back of her legs. The pain is aggravated by standing for long period of time and lifting heavy objects. MRI revealed L4-5 disc protrusion with L-5 nerve compression. What is the most appropriate first action to be taken by the physiotherapist in this case?
 - A. Assessment of range of motion
 - B. Take appropriate patient history
 - C. Assessment of muscle power
 - D. Design treatment plan

Answer Key: The answer is **B**

Explanation - Taking a thorough patient history is the first and most important step. It helps the physiotherapist understand the nature of the condition, identify any red flags, and ensure that it is safe to proceed with physical assessment and treatment.

2. A 50-year-old male patient came to a physiotherapy clinic with a complaint of pain on the pelvic and sacroiliac joint. On examination, tenderness was noticed on both sacroiliac joints was noticed. Then, he was diagnosed with sacroiliac joint dysfunction. What is the most commonly used special test to examine this case?
 - A. Patric's test
 - B. Straight leg raise test
 - C. Phalen's test
 - D. Apprehension test

Answer Key: The answer is **A**

Explanation: Patrick's test (FABER test) is the most used test for sacroiliac joint (SIJ) dysfunction. The test involves placing the leg in a Flexion, Abduction, and External Rotation (FABER) position. Pain in the SI joint region during this test suggests SI joint involvement. Straight Leg Raise test Used to assess for lumbar disc herniation or sciatica, not SIJ dysfunction. It checks for nerve root irritation, especially L4-S1. Phalen's test Used to diagnose carpal tunnel syndrome. It involves wrist flexion to provoke median nerve symptoms. Not related to SIJ. Apprehension test Typically used to evaluate shoulder instability or dislocation, particularly anterior instability. Not applicable to pelvic or SI joints.

3. A 55-year-old woman, who has seven children, came to neurosurgery OPD with a complaint of chronic lower back pain and urine incontinence. The neurosurgeon then consulted for physiotherapy intervention after excluding red flags. On examination, the physiotherapist found tenderness on lumbar region and weakness of the pelvic floor muscles.

What is the most appropriate initial treatment for this case?

- A. Lumbar Traction
- B. Fitness exercise
- C. Kegel's exercise
- D. Cervical traction

Answer Key: The answer is **C**

Explanation: Kegel's exercise is the most appropriate initial treatment for pelvic floor muscle weakness and urinary incontinence. It strengthens the pelvic floor muscles and help improve bladder control, especially in women with a history of multiple childbirths. Lumbar traction Used for spinal decompression in cases like disc herniation or nerve root compression, but not appropriate as initial treatment for pelvic floor weakness or incontinence. Fitness exercise General fitness is helpful in long-term back pain management, but it's not specific or targeted for the primary issue here pelvic floor weakness and incontinence. Cervical traction Used for neck (cervical spine) issues like cervical radiculopathy not related to this patient's lumbar or pelvic complaints.

4. A one and half-year-old girl was brought to a physiotherapy OPD with a complaint of developmental delay. The physiotherapist wants to assess the child to provide a definitive diagnosis and plan of treatment.

What are the optimal assessment parameters to be used to determine the child's diagnosis?

- A. Range of motion assessment, and sensory, speech, and fine motor skill evaluation
- B. Observation for deformity, muscle power, muscle length and cognitive assessment
- C. Gross motor, fine motor, speech and comprehensive evaluation of functional abilities
- D. Observation for deformity, manual muscle testing, tone and sensory assessment

Answer Key: The answer is **C**

Explanation: This option covers all key developmental domains including Gross motor (e.g., crawling, walking), Fine motor (e.g., grasping, object manipulation), Speech and language (important for communication milestones) and Functional abilities (e.g., feeding, dressing, play)

Range of motion assessment, sensory, speech, and fine motor skill evaluation are important but not sufficient for a full developmental assessment. It lacks gross motor and functional assessments, which are essential in identifying developmental delays in young children.

Observation for deformity, muscle power, muscle length, and cognitive assessment are not appropriate for toddlers, and it misses key developmental areas like gross and fine motor skills.

Observation for deformity, manual muscle testing, tone and sensory assessment focuses on neuromuscular aspects and lacks evaluation of developmental milestones and functional skills, which are critical in this child.

5. A pediatric physiotherapist performing physical examination and postural analysis on a 12-year-old girl who had idiopathic scoliosis found out a classic rib hump on the right thoracic spine during forward bend test. The physiotherapist also noticed a lateral thoracic curvature with concavity to the left and a slight compensatory tilting in the lumbar region with concavity to the right. There was also shortening of paraspinal muscles in the left thoracic and right quadrates lumborum.

What is the best possible diagnosis for this child?

- A. Left thoracic right lumbar structural scoliosis
- B. Right thoracic left lumbar nonstructural scoliosis
- C. Left thoracic right lumbar nonstructural scoliosis
- D. Right thoracic left lumbar structural scoliosis

Answer Key: The answer is **D**

Explanation: Rib hump on the right thoracic spine during forward bend test is a hallmark sign of structural scoliosis, due to vertebral rotation that matches the clinical findings; right thoracic curvature (with rib hump), left lumbar curve (compensatory) and structural scoliosis (confirmed by rib hump and rotation)

Left thoracic right lumbar structural scoliosis is incorrect as the thoracic curve is right, not left. Right thoracic left lumbar non-structural scoliosis is incorrect as the presence of a rib hump indicates structural scoliosis.

Left thoracic right lumbar non-structural scoliosis is incorrect as both the curve direction and scoliosis type do not match the findings.

6. A nine-year-old boy diagnosed with Duchenne muscular dystrophy with marked joint contracture, weakness of leg and trunk muscles, impaired pulmonary function and functional ability was brought to a physiotherapy clinic for his follow-up. After performing reassessment, the physiotherapist designed a SMART treatment goal.

What is the most likely treatment goal that fits for this child?

- A. Correcting of contracture with casting
- B. Improving muscle strength to grade 5/5
- C. Treating pulmonary capacity until normal
- D. Preventing deformity and maintaining functioning

Answer Key: The answer is **D**

Explanation: The physiotherapy goal must be realistic, functional, and progressive—aligned with SMART criteria (Specific, Measurable, Achievable, Relevant, Time-bound). DMD is a progressive genetic muscle-wasting disorder. At age nine, most children with DMD have: significant muscle weakness (especially lower limbs and trunk), joint contractures and declining pulmonary function. Preventing deformity and maintaining functioning is Realistic and achievable and the main physiotherapy goals in DMD: Slow progression of contractures and deformities, maintain mobility and independence as long as possible, and support respiratory health.

7. A 48-year-old patient diagnosed with post poliomyelitis syndrome was referred to a physiotherapist for a follow-up evaluation and management. The patient complained of fatigue which commonly occurs in the afternoon, weakness in the right lower limb and breathing difficulties.

What are the best exercises to treat the patient's condition?

- A. High intensity resistance training for the affected lower limb and chest exercises
- B. Chest exercises, TENS for the affected lower limb, and functional re-education
- C. Motor control re-education, sensory re-education, and chest exercise
- D. Gait training, provision of ankle foot orthosis and lumbar support

Answer key: The answer is **B**

Explanation: Chest exercises, TENS for the affected lower limb, and functional re-education. This approach addresses the main symptoms of PPS as chest exercises improve respiratory efficiency, TENS helps manage muscle fatigue and discomfort and functional re-education focuses on safe movement patterns to conserve energy and prevent overuse.

8. A 65-year-old male patient was admitted to a hospital with a complaint of muscle weakness. Even though multidisciplinary team was involved on the assessment, the professionals faced difficulty in understanding the case in detail. After a detail discussion and investigation, they agreed that the patient's case is not common and mostly affects males between the age of 50 and 70. The disease progressively damaged the efferent nerve cells of central nervous system, and the prognosis is poor.

What is the most likely diagnosis of this case?

- A. Guillain–Barre syndrome
- B. Cerebrovascular accident
- C. Motor neuron diseases
- D. Multiple sclerosis

Answer key: The answer is C

Explanation: Motor Neuron Disease (MND) are progressive, degenerative disorders of the motor neurons (efferent CNS cells) that is most common in men aged 50–70 with progressive muscle weakness, wasting, respiratory issues and poor prognosis especially in Amyotrophic Lateral Sclerosis (ALS). Because it best fits the age group, male predominance, progressive CNS motor neuron damage, and poor prognosis described in the case. These features strongly indicate MND specifically ALS, the most common type of MND.

9. A 70-year-old female patient came to a physiotherapy clinic with complaints of weakness of extremities and tremor on her left hand. Upon examination, the physiotherapist found muscle weakness on the left upper and lower limb with a muscle manual testing of 2 out of 5 and 3 out of 5 respectively. The gait pattern of the patient is Trendelenburg, with a strong lateral lean to the left side of her body.

What is the most appropriate treatment plan for this case?

- A. To increase the hip abductors muscle strength from 2 to 3 within two weeks
- B. To increase the hip flexors muscle strength from 2 to 3 within two weeks
- C. To increase the quadriceps muscle strength from 2 to 3 within two weeks
- D. To increase the hip adductors' muscle strength from 2 to 3 within two weeks

Answer Key: The answer is A

Explanation: Increase the hip abductors muscle strength from 2 to 3 within two weeks will directly address the Trendelenburg gait and lateral lean, improving stability and function. Hip abductor weakness causes Trendelenburg gait and lateral lean. Strengthening hip abductors improves pelvic stability during stance and gait.

10. A 67-year-old man who had a history of outstretched fall down injury seven months ago presented with a complaint of pain, swelling, and limited movement of his affected part. The radiological finding revealed an impacted type of fracture on the affected part.

What is the most likely diagnosis of this patient?

- A. Supracondylar fracture
- B. Shaft of humerus fracture
- C. Proximal humerus fracture
- D. Humerus neck fracture

Answer key: The answer is D

Explanation: Proximal humerus fracture is the most common fracture in elderly people after a fall on an outstretched hand or direct shoulder trauma. Impacted fractures are common here due to the cancellous bone in the proximal humerus. The symptoms of pain, swelling, and limited shoulder movement match this diagnosis. Because it fits the patient's age, injury mechanism, clinical features, and radiological findings best.

11. A 40-year-old woman came to physiotherapy outpatient clinic with a complaint of morning stiffness, bilateral symmetrical symptoms and swelling of PIP, MCP and MTP joints which is getting worse through time since a year ago. The radiological findings showed bony erosion and localized decalcification of joints.

What is the most likely diagnosis of this patient?

- A. Osteoarthritis
- B. Rheumatoid arthritis
- C. Ankylosing spondylitis
- D. Juvenile idiopathic arthritis

Answer key: The answer is **B**

Explanation: Rheumatoid arthritis has Classic presentations of: Bilateral symmetrical joint involvement affecting small joints (PIP, MCP, MTP) with morning stiffness lasting >1 hour. On radiology bony erosions and periarticular osteopenia (localized decalcification) match patient's presentation.

12. A 25-year-old 3rd trimester pregnant woman came to a physiotherapy OPD with a diagnosis of carpal tunnel syndrome. After the assessment, a physiotherapist wants to prescribe orthosis for the client.

What is the most appropriate orthosis in the management of this patient?

- A. Elbow orthosis
- B. Wrist hand orthosis
- C. Shoulder elbow orthosis
- D. Shoulder elbow wrist orthosis

Answer key: The answer is **B**

Explanation: The most appropriate orthosis for CTS Wrist hand orthosis which keeps the wrist in a neutral or slightly extended position, reduces pressure on the median nerve in the carpal tunnel, provides symptom relief, especially at night and commonly prescribed during pregnancy when CTS is frequent due to fluid retention. Because it effectively immobilizes the wrist in a neutral position, relieving median nerve compression in carpal tunnel syndrome, especially in pregnancy.

13. A 38-year-old woman on her 15th postpartum day came to a hospital with complaints of difficulty to control her urine during coughing, laughing, sneezing and while she was lifting heavy objects. She denies history trauma and she had normal vaginal delivery with normal labour duration.

What is the most likely diagnosis?

- A. Urinary urgency and frequency
- B. Stress urinary incontinence
- C. Urge urinary incontinence
- D. Mixed urinary incontinence

Answer Key: The answer is **B**

Explanation: In Stress urinary incontinence (SUI) Leakage of urine occurs with increased intra-abdominal pressure (coughing, sneezing, laughing, lifting), due to pelvic floor muscle weakness or damage often seen postpartum after vaginal delivery. Because the symptoms of leakage during activities that increase abdominal pressure postpartum are classic for stress urinary incontinence.

14. A 57-year-old male cigarette smoker who had chronic stable obstructive pulmonary disease visited physician with a complaint of coughing, expectoration, and dyspnea. He was managed with anti-cholinergic drugs and then consulted for pulmonary rehabilitation. What is the most appropriate physiotherapy management for the dyspnea?
- A. Huffing and coughing
 - B. Manual Percussion
 - C. Pursed lip exercise
 - D. Riding bicycle

Answer Key: The answer is **C**

Explanation: Pursed lip breathing technique helps slow down expiration and prevent airway collapse in COPD, improves ventilation efficiency and reduces shortness of breath and commonly taught in pulmonary rehab for dyspnea management. Because it effectively reduces dyspnea by improving airway pressure and ventilation in COPD patients.

15. A 26-year-old female soccer player has got contact injury on her right knee joint. She falls with right foot in a plantar flexion position; right knee in slight flexion and moving into full extension and external rotation. She complained about swelling soon after the injury. On examination, Lachman test was positive whereas anterior and posterior drawer tests were negative. What is the most likely diagnosis of this case?
- A. Anterior cruciate ligament sprain
 - B. Posterior cruciate ligament sprain
 - C. Medial collateral ligament sprain
 - D. Lateral collateral ligament sprain

Answer Key: The answer is **A**

Explanation: Anterior cruciate ligament (ACL) sprain is correct as Lachman test is the most sensitive test for ACL injuries. Positive Lachman test with negative anterior drawers may indicate partial tears or early presentation. Mechanism (forced extension and external rotation) is typical for ACL injury and rapid swelling suggests intra-articular injury like ACL tear.

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List of contributors

Mr. Endalkachew Tsedal	MoH/ HHRIPR LEO
Dr. Meron Yakob	MoH/ HHRIPR Exam Development and Administration Desk Lead
Ermias G/Yohannes	MoH/ HHRIPR
Dr. Wudasie Teshome	MoH/ HHRIPR
Dr. Melese Achamo	MoH/ HHRIPR
Tsedale Tafesse	MoH/ HHRIPR
Mekonnen Desie	MoH/ HHRIPR
Abera Bezabih	MoH/ HHRIPR
Fikadie Dagnaw	MoH/ HHRIPR
Melkamu Gereme	MoH/ HHRIPR
Tadele Asefa	MoH/ HHRIPR
Neway H/Michael	MoH/ HHRIPR
Mebrahtu Tarekegn	MoH/ HHRIPR
Milkoman Ayele	MoH/ HHRIPR
Bethelhem Shikabew	MoH/ HHRIPR
Amanuel Tesfay	MoH/ HHRIPR

Contact Us

Ministry of Health – Ethiopia

Street Address: - 1234 Sudan Street, Addis Ababa – Ethiopia

Telephone: +251-118275936

Hotline service: - 952

Website: <https://www.moh.gov.et>

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